



PROCEDURE

Medication: Clozapine Management by General Practitioners

Scope (Staff):	North Metropolitan Health Service Mental Health (NMHS MH) Staff: Nursing/Medical
Scope (Area):	NMHS MH Community Mental Health Clinics

1. Aim

To provide a procedure for NMHS MH Community Mental Health Clinics (CMHC) to manage the transition of stable clozapine patients to their General Practitioner (GP) and provide guidance to GPs on managing clozapine as part of a shared care arrangement.

2. Background

The clozapine GP shared care model first came about when regulations changed allowing GPs to prescribe maintenance clozapine under the guidance of a psychiatrist. The aim of this model is to optimise care for patients with schizophrenia who are stabilised on clozapine to help them transition back to GP managed community-based care. The patient remains under the supervision and direction of the treating psychiatrist, and must be reviewed by them at least every 12 months, ongoing.

GPs are well placed to provide ongoing care to people taking clozapine. Necessary components include agreed monitoring protocols, and prescribing responsibilities for prophylaxis and treatment of clozapine related adverse effects. Good communication between clozapine coordinators, GPs, pharmacists and patients is essential.

It is important to note that not all clozapine patients on maintenance therapy are suitable for GP prescribing. It is the characteristic of the patient receiving clozapine rather than the duration of clozapine therapy which must be the determining factor in any transition of care.

3. Definitions

FBC: Full Blood Count

WBC: White Blood Cell Count

ANC: Absolute Neutrophil Count

PBS: Pharmaceutical Benefits Scheme

4. Procedure for transition to the GP

4.1 Inclusion criteria

To be considered eligible for transition to the care of the GP, a patient must meet the following criteria:

-
1. Taking clozapine for at least 12 months
 2. Attended the CMHC for a minimum of 6 months
 3. Managed by the coordinator-led clozapine clinic prior to transfer
 4. Good history of adherence with clozapine
 5. Clinically stable with no mental health hospital admissions in the previous 12 months
 6. Able to independently attend blood tests, appointments and other investigations, or adhere to the requirements of the treatment plan with support from family or supported accommodation if the psychiatrist, carer and GP agree
 7. No or low case management needs
 8. Stable dose of clozapine for at least 3 months
 9. Patient and GP agreeable to transition
 10. Stable accommodation and readily contactable
 11. Have an appointed support person to contact in an emergency or if patient cannot be contacted.

Other considerations:

- History of overdose
- History of red range (**WBC<3.0 x 10⁹/L or ANC<1.5 x 10⁹/L**) blood result within the previous 12 months
- Patients with benign ethnic neutropenia (BEN) who may have lower ranges for WBC and ANC approved by the Clopine® haematologist.
- Ability to bear the financial loss of access to voucher system e.g. for Blister Packing

Once a patient has been identified as suitable, the clozapine coordinator will liaise with the patient and GP to facilitate the transition to the shared care arrangement.

The psychiatrist will write a detailed referral letter (template letter available in Best Practice software) and provide a handover document for the GP. The handover document will detail relevant history; e.g. physical health monitoring results, recent medical reviews, psychiatric history, current medication and adverse reaction lists, current pharmacy and any other relevant information specific to the patient.

The clozapine coordinator will attend the patient's first clozapine appointment to provide further handover.

4.2 Identification of a GP

For a GP to take over clozapine management they must undertake the following measures:

- Register with ClopineCentral™
- Agree to adhere to Clopine® protocols
- Be available to review the patient every 4 weeks
- If taking leave, ensure cover with a Clopine® registered GP



- Be willing to bulk bill appointments
- Refer the patient for psychiatrist review at the CMHC every 12 months and more frequently if clinically indicated
- Create a mental health care plan if appropriate.

The GP must refer back to the relevant CMHC in the circumstances below. In urgent situations, the GP should telephone the clinic, otherwise the usual contact procedure will apply.

- Blood tests are in the:
 - Red range **WBC<3.0 x 10⁹/L or ANC<1.5 x 10⁹/L**
- Clozapine therapy interruption of greater than 2 days
- Significant deterioration in mental state or concerns about mental state
- Patient/carer requests review
- Evidence of non-adherence with medication, blood testing or not attending appointments
- Unable to cover a clozapine appointment with a replacement GP in an acceptable time frame to meet clozapine protocol
- Clozapine dose changes - the GP is required to consult with the CMHC psychiatrist if they believe a dose adjustment is indicated for any reason.

GP should liaise with the CMHC when:

- Blood test is in the:
 - Amber range **WBC 3-3.5 x 10⁹/L or ANC1.5-2.0 x 10⁹/L**
- Changes to smoking habits - This can result in significant changes in clozapine levels. Advisable to check clozapine level and consult with psychiatrist if adverse effects develop and dose needs to be reduced.
- Pregnancy, planning to conceive, breastfeeding.

4.3 First appointment

The first appointment between the patient and GP for clozapine management will be attended by the clozapine coordinator, to ensure that the clinical handover has occurred, to give the patient and GP any further information required, and to ensure that the patient is booked in for their monthly GP appointments.

At this appointment administrative staff at the GP practice should organise a referral for the patient's annual appointments with the psychiatrist so they are not missed. It is a requirement that the patient is reviewed by the psychiatrist at a minimum of annually.

4.4 Subsequent appointments

GP appointments must be performed at least every four weeks

Each appointment should be made in advance, preferably at the patient's previous appointment.

At each appointment, the GP will review blood tests, ensuring that WBC and ANC are within the acceptable range.

The GP is also to assess for:

- Signs of infection (flu like symptoms, fever, malaise and myalgia)
 - Signs of cardiac illness (tachycardia, chest pain, shortness of breath)
 - Drug interactions.
- a. Physical health parameters ([Appendix 3](#) Form A)
- Blood Pressure
 - Heart rate
 - Temperature
 - Weight
 - Waist circumference
 - Ask about adverse effects
 - Review caffeine consumption and smoking habits (caffeine can increase clozapine levels, whereas smoking can reduce clozapine levels)
 - Discuss bowel habits and ensure constipation is managed
- b. Review mental state
- c. Review medication adherence

4.5 Physical Health Monitoring

Physical health monitoring is essential to ensure safe management of patients taking clozapine. The minimum requirements are outlined in the suggested tools Form A ([Appendix 3](#)) and Form B ([Appendix 4](#)):

- [Clozapine Monitoring Form Part A](#) – Four weekly monitoring for haematological, gastrointestinal, cardiovascular and lifestyle habits. Also complete the:
 - [GASS for clozapine](#) side effect tool ([Appendix 2](#))
 - Constipation Assessment ([Appendix 1](#))
 - Clozapine induced Gastrointestinal Hypomotility (CIGH) is increasingly recognised as a highly prevalent and potentially fatal adverse effect of clozapine. Active monitoring, including direct questioning, is essential.
- [Clozapine Monitoring Form Part B](#) Metabolic and cardiac monitoring to be checked when recommended.

These monitoring tools are available for the GP to use as is, or as a reference for a local monitoring system.

5. Prescribing Clozapine

Clozapine must be prescribed only in the quantity required to ensure the patient has enough medication until their next blood test (usually 28 days).

Do NOT prescribe the maximum PBS quantity of 200 tablets per strength.

Clozapine can be prescribed at each appointment with the PBS streamline code or it is possible to obtain phone authority to prescribe up to five repeats for monthly dispensing to reduce the need



to prescribe at each appointment.

The pharmacy will not dispense clozapine without a current (within 48 hours) blood test signed by the doctor (or results entered into ClopineCentral™) and will retain the repeats.

Note: Clopine® Brand must be prescribed for NMHS MH patients as this is the monitoring system used (no brand substitution). Other brands of clozapine (e.g. Clozaril®) have separate monitoring systems that do not communicate with the ClopineCentral™ monitoring system.

6. ClopineCentral™ Terminology

For a clozapine patient whose clozapine is being managed by their GP the ClopineCentral™ set-up will be as follows:

- The CMHC will be the overseeing “Clopine® Centre”
- The GP will be registered as a “Clinic” of the relevant CMHC Clopine® Centre
- The community pharmacy dispensing clozapine will also be registered as a “Clinic” under the relevant CMHC Clopine® centre as is currently the case for Clopine® dispensing pharmacies in the NMHS Catchment.

This set up will allow the clozapine coordinator of the relevant CMHC to identify if either a GP or pharmacy is having difficulty with the Clopine® process and assist with follow up. It will also limit the GP and pharmacy to have access only to the information of the patients they are responsible for. This will also make it simpler for the CMHC to access patient information should consultations with the GP be needed and to allow simple transfer if management needs to be taken back by the CMHC due to any issues encountered.

7. Community Mental Health Clinic Review

At a minimum, the GP will refer their clozapine patients to the relevant CMHC for an annual review with a psychiatrist. Additional assessments can be requested if required.

8. Roles and Responsibilities

8.1 Psychiatrist

As a minimum requirement a psychiatrist will review the patient at the CMHC every 12 months.

CMHC psychiatrists are also available during the working week for GPs to discuss any issues relevant to clozapine patients being managed by them. Each mental health clinic has a clozapine coordinator who will triage calls from GPs and where required, will organise urgent contact with a consultant psychiatrist for review and advice.

The psychiatrist at the CMHC must take back care of the patient at any time if there is a significant deterioration in mental state.



8.2 GP

Responsibilities of the GP include:

- Ensuring required monitoring is completed and act on abnormal results as necessary.
- Prescribing the maintenance dose of clozapine after each blood test.
- Providing the patient or pharmacy with:
 - A copy of the blood results (WBC, ANC) with the prescription/s; or
 - The prescription/s with a note that blood results have been entered into ClopineCentral™
- Communicating with the treating psychiatrist if there are any concerns/changes to circumstances etc.
- Ensuring the patient has a review with the psychiatrist at least annually.
- Collating and sending relevant annual pathology results and monitoring (e.g. ECG, metabolic monitoring, clozapine level, echocardiogram if required) to the clozapine coordinator every 12 months to coincide with annual psychiatrist review.

8.3 Clozapine Coordinator

Each CMHC has a clozapine coordinator who will maintain oversight of GP-managed clozapine patients through the administration of the ClopineCentral™ database. The clozapine coordinator will also be available to provide support, including but not limited to:

- Triaging calls from GPs managing clozapine and involving the psychiatrist when required.
- Providing education and support on clozapine protocol to GPs & community pharmacists.
- Managing Clopine® registration of new personnel, GPs and pharmacies on the ClopineCentral™ site.
- Follow up on overdue blood tests.
- Prior to the annual appointment collate monitoring data (blood tests including clozapine level, metabolic screen, latest ECG and echocardiogram if required).
- Attend annual review appointment with psychiatrist if required, where possible.

8.4 Community Pharmacy and Dispensing Pharmacist

The process of transfer of care to the GP will include identification of the patient's preferred community pharmacy as this may change when the patient no longer needs to attend the clinic for clozapine management. The community pharmacy will be advised of the transfer to GP shared care prior to the first GP appointment to ensure the required registrations are in place.

The pharmacy and all dispensing pharmacists must:

- Be registered with ClopineCentral™
- Adhere to Clopine® protocols
- Only dispense clozapine once blood results have been reviewed and entered on ClopineCentral™
- Liaise with the GP for clozapine related matters.



-
- Contact the patient if they do not collect clozapine when expected. If not able to contact the patient the pharmacist will contact the GP to inform them. This is important because if the patient misses more than two consecutive days of clozapine then they need to be re-titrated from a low dose.

8.6 Clozapine Clinic Clinical Pharmacist

- Assist in provision of education to GPs, community pharmacies and CMHC clinicians.
- Available for brief consultations.
- Contact point for clozapine related queries and troubleshooting.

8.7 Patient taking clozapine

For transition of a patient taking clozapine to a share care agreement with their GP the patient must agree to actively participate in:

- Attending a pathology centre for blood test the morning or day before their appointment with their GP (every 28 days or as required by protocol)
- Attending appointments with their GP every 28 days – or as required by protocol,
- Attending their nominated pharmacy to collect clozapine prescription
- Attending review appointments at their CMHC every 12 months
- Contacting their GP if they have any concerns in between appointments particularly if they experience constipation, sore throat or feeling unwell.

9. Troubleshooting

9.1 Non-attendance for appointments

If a patient does not attend an appointment as required, the following steps should be taken by the GP:

- **Step 1**

Contact the patient or nominated support person to arrange another appointment.

Continue clozapine management if the patient attends the appointment and receives a blood test, as long as any interruption in therapy is less than 2 days.

- **Step 2**

If the patient is unable to attend an appointment or has missed more than 2 days of clozapine, refer patient back to the CMHC as the CMHC will need to facilitate clozapine dose re-titration.

If the patient is not contactable or unwilling to recommence clozapine refer back to the CMHC for further management.



9.2 No FBC result

- The GP should identify the reason the patient did not receive a blood test or why the results are not back. If satisfied that the patient agrees to have their blood test the GP can continue the clozapine assessment as usual.
- If the patient will run out of medication before a blood test can be done the GP can apply for a “Monitoring & Supply Change” on ClopineCentral™ (apply on-line or on the phone). This will allow enough supply of clozapine until the blood test is done and reviewed. Refer to the [ClopineCentral™ Protocol](#).
- If the patient receives a late blood test after their monthly appointment and the GP is confident there has not been an interruption to therapy of more than 2 days, a prescription can be issued.

9.3 Nonadherence with clozapine

The GP should contact the CMHC and refer the patient back to the clinic for review and to decide on further management.

CMHC will liaise with the GP and the patient on whether they will take back management of patient’s clozapine or return management to the GP. This will be on a case by case basis.

10. Contact Information

Wanneroo Adult Community Mental Health Service

2 Caffagio Crescent, Wanneroo, 6065

Phone (08) 9406 7100 Fax (08) 9406 7190

Operating Hours Monday to Friday 08:30 - 16:30

Butler Community Mental Health Service

81 Exmouth Drive, Butler, 6036

Phone (08) 6372 1500 Fax (08) 6372 1599

Mirrabooka Community Mental Health Service

Unit 1, 20 Chesterfield Road, Mirrabooka 6061

Phone (08) 9344 5400 Fax (08) 9345 2631

Operating Hours Monday to Friday 08:30 - 16:30

Osborne Community Mental Health Service

Block H, Osborne Park Campus, Osborne Place, Stirling 6021

Phone: (08) 6457 8350 Fax (08) 6457 8388

Operating Hours Monday to Friday 08:30 - 16:30

Lower West Community Mental Health Service

303 Rokeby Road, Subiaco, 6008

Phone (08) 9489 7200 Fax (08) 9382 4171

Operating Hours Monday to Friday 08:30 - 16:30

Mental Health Emergency Response Line (MHERL)

Telephone 1300 555 788 Operating Hours 24/7

Clozapine Clinic Clinical Pharmacist

Telephone: (08) 6159 6677

Opening hours Monday to Wednesday 8:30 – 16:30

Community Liaison Pharmacist NMHS MH

Telephone: (08) 6159 6595

Email: NMAHSCLP@health.wa.gov.au

Operating Hours Monday to Friday 08:30 - 16:30

State-wide Psychotropic Drug Information Centre

Telephone: (08) 6159 6400

Email: DrugInformation.Graylands@health.wa.gov.au

Operating Hours Monday to Friday 08:30 - 16:30

On-Call Pharmacist NMHS MH

Telephone: NMHS Telecommunications (08) 6159 6600

Operating Hours Monday to Friday 16:30 - 08:30,

Saturday, Sunday & Public Holidays 24 hours

Related internal policies, procedures and guidelines
NMHS MH Medication: Clozapine Prescribing and Monitoring NMHS MH Medication: Clozapine Coordinator – Clozapine Clinic

References
Clinical Guidelines for the Physical Care of Mental Health Consumers Australian Prescriber: Clozapine in primary care 2017 ClopineCentral™ Protocol Therapeutic Guidelines > Gastrointestinal > Constipation > Functional gastrointestinal disorders > functional constipation

Useful resources (including related forms)

[GASS for Clozapine](#)

[Clozapine Monitoring Form Part A](#)

[Clozapine Monitoring Form Part B](#)

[Choice and Medications](#)

Policy Sponsor	Chief Pharmacist, NMHS MH				
Policy Contact	Chief Pharmacist, NMHS MH				
Date First Issued:	26/02/2019	Last Reviewed:	10/10/2022	Review Date:	10/10/2025
Approved by:	NMHS MH Drugs and Therapeutics Committee (DTC)			Date:	04/10/2022
Endorsed by:	Director of Clinical Services MHPHDS			Date:	10/10/2022
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety		
National Standards for Mental Health Services	☒ Std 8: Governance, leadership and management		☒ Std 9: Integration ☒ Std 10: Delivery of Care		
Chief Psychiatrist's Standards for Clinical Care:	Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	Version	Author	Notes
25/02/2019	1.0	MHEG	Endorsed out of session via email
26/02/2019	1.0	Executive Director, MHPHDS	Submission form
26/02/2019	1.0	Chief Pharmacist, NMHS MH	Initial Endorsement
05/10/2022	2.0	Chief Pharmacist, NMHS MH	New template; Information regarding WA Clozapine Guidelines removed, at this stage no longer available. Updated information and hyperlinks.

This document can be made available in alternative formats on request for a person with a disability.

© North Metropolitan Health Service 2022

Copyright to this material is vested in the State of Western Australia unless otherwise indicated. Apart from any fair dealing for the purposes of private study, research, criticism or review, as permitted under the provisions of the *Copyright Act 1968*, no part may be reproduced or re-used for any purposes whatsoever without written permission of the State of Western Australia.

Appendix 1. Constipation Assessment Tool

The normal frequency of bowel motions can vary between individuals from 3 times per day to 3 times per week.

Therefore, functional constipation should not be defined by stool frequency alone.

Constipation should be assessed using the following criteria in conjunction with the Bristol Stool chart.

Type 1	Type 2	Type 3	Type 4	Type 5	Type 6	Type 7
						
Separate lumps, hard pebbles	Hard lumps forming a sausage	Sausage-shaped, surface looks like cracked paint	Soft sausage-shape, smooth surface	Separate soft blobs	Soft mushy stool almost fluffy in appearance	Fluid only no solid pieces

Constipation can be confirmed when 2 or more of the following are present:

- two or fewer motions per week
- straining at more than 25% of defecations
- a feeling of incomplete evacuation at 25% or more defecations
- hard lumpy stools
- sensation of anorectal obstruction or blockage for at least 25% of defecations
- manual manoeuvres needed to facilitate at least 25% of defecations (e.g. digital evacuation, support of the pelvic floor).

It is important to review current medication therapy and ask about the use of over the counter medications, complementary medicines and laxatives to assist in identifying possible causes of constipation and determine appropriate management

Other common issues that could be exacerbating or causing constipation should also be assessed including:

- review other medication that could be causing constipation (particularly anticholinergics)
- inadequate dietary fibre
- dehydration
- inappropriate bowel habits (e.g. ignoring the urge to defecate)
- inadequate physical activity (e.g. from decreased mobility)
- change in environment (e.g. holiday, entry into a residential aged care facility)
- painful anorectal disorders (e.g. haemorrhoids, anal fissure)
- loss of muscle power caused by other medical conditions (e.g. chronic obstructive pulmonary disease).

Red flags - Signs and symptoms that warrant immediate medical attention include moderate to severe abdominal pain, abdominal distension, vomiting, overflow diarrhoea and signs of sepsis. Reference:

Appendix 2. GASS for Clozapine

GASS for Clozapine

Name: _____ **Current Medications:** _____
Date: _____
Caffeine intake:cups/day _____
Smoker: Y / Ncigarettes/day _____

Has there been a recent change in your smoking habit?: Increase/Decrease by.....cigarettes/day

This questionnaire is being used to determine if you are suffering from excessive side effects from your medication.
Please put a tick in the column which best indicates how often or how severely you have experienced the following side effects.

Over the past week :		Never	Once	A few times	Everyday	Tick if severe or distressing
1	I felt sleepy during the day					
2	I felt drugged or like a zombie					
3	I felt dizzy when I stood up or have fainted					
4	I have felt my heart beating irregularly or unusually fast					
5	I have experienced jerking limbs or muscles					
6	I have been drooling					
7	My vision has been blurry					
8	My mouth has been dry					
9	I have felt sick (nauseous) or have vomited					
10	I have felt gastric reflux or heartburn					
11	I have had problems opening my bowels (constipation)					
12	I have wet the bed					
13	I have been passing urine more often					
14	I have been thirsty					
15	I have felt more hungry than usual or have gained weight					
16	I have been having sexual problems					

I have also experienced:

(please write down any other side effects **OR PHYSICAL PROBLEMS OR COMPLAINTS** that you may have experienced over the **past week**)

17	
18	
19	
20	

Adapted from the Glasgow Antipsychotic Side-effect Scale© 2007 by St. John of God Hospital and South London and Maudsley Trust

Waddell, Taylor and Hynes 2012
DT,FG,AA, PH, RD comments

Appendix 2. GASS for Clozapine

Staff Information

1. Allow the service user to fill in the side-effects scale themselves. All questions relate to the previous week.

2. **Scoring**

0 Points	"Never"
1 point	"Once"
2 points	"A few times"
3 points	"Everyday"

3. **Results**

0-16	absent/mild side-effects
17-32	moderate side-effects
33-48	severe side-effects

4. **Side-effects covered include:**

1-2	Drowsiness and sedation
3	Postural hypotension
4	Tachycardia
5	Myoclonus
6	Hypersalivation
7-8	Anticholinergic side-effects
9-10	Gastrointestinal side-effects
11	Constipation
12	Nocturnal enuresis
13-14	Screening for diabetes mellitus
15	Weight gain
16	Sexual dysfunction

5. The column relating to the severity/distress experienced with a particular side effect is not scored, but is intended to inform the clinician of the service user's views and condition.
6. Questions 17 to 20 invite the service user to report any other side-effects or problems not already mentioned. These questions should not be scored but may instigate a discussion with the service user if clinically appropriate.

Adapted from the Glasgow Antipsychotic Side-effect Scale© 2007 by St. John of God Hospital and South London and Maudsley Trust

Waddell, Taylor and Hynes 2012
DT,FG,AA, PH, RD comments

Appendix 3. Clozapine Monitoring Form – Part A

CLOZAPINE MONITORING FORM – PART A (to be used after WA clozapine initiation and titration chart)		Psychiatrist / Treating Doctor:				SURNAME		UMRN		
		Monitoring clinic / GP:				GIVEN NAMES		DOB	GENDER	
		Dispensing Pharmacy:				ADDRESS		POSTCODE		
		WARD _____		DOCTOR _____				TELEPHONE		
Patient Contact Number: _____		Height: _____ m		Date clozapine initiated: ____ / ____ / ____ 18 week clozapine completion date: ____ / ____ / ____						
Clozapine patient number (CPN): _____		Blood Group: _____								
CLINICAL REVIEW		Baseline prior to clozapine	Date	Date	Date	Date	Date	Date	Date	Date
Baseline then weekly first 18 weeks, then every 28 days	Dosage (mg): AM									
	Dosage (mg): PM									
	Blood pressure – lying									
	Blood pressure – standing									
	Temperature (°C)									
	Heart rate (bpm)									
	Weight (kg)									
	Waist circumference (cm)									
	Cigarettes / day									
	WBC > 3.5 x 10 ⁹ /L AND NC > 2.0 x 10 ⁹ /L		Green	WBC > 3.0 – 3.5 x 10 ⁹ /L AND/OR NC 1.5 – 2.0 x 10 ⁹ /L		Amber	WBC < 3.0 x 10 ⁹ /L AND/OR NC < 1.5 x 10 ⁹ /L		Red	
FBC or WBC and differentials x10 ⁹ /L	WBC NC									
Signs of infection (flu like symptoms, fever, malaise and myalgia)										
Signs of Cardiac illness (Tachycardia, chest pain, shortness of breath)										
GASS for Clozapine completed										
Constipation Assessment completed										
DETAILS OF PERSON COMPLETING THIS FORM These are suggested guidelines only, refer to the treating psychiatrist for individual monitoring requirements										
Signature or initials										
Name (Please print)										

Appendix 4. Clozapine Monitoring Form – Part B

CLOZAPINE MONITORING FORM – PART B (to be used after WA clozapine initiation and titration chart)												SURNAME				UMRN							
												GIVEN NAMES				DOB		GENDER					
												ADDRESS								POSTCODE			
																TELEPHONE							
WARD _____		DOCTOR _____																					
Please indicate N (Normal) or A/N (Abnormal) in relevant white box once test completed (grey boxes are not mandatory but repeat tests if clinically indicated)																							
CLINICAL REVIEW REMINDERS	Baseline	1 week	2 weeks	3 weeks	4 weeks	5 weeks	6 weeks	12 weeks	6 months	12 months	18 months	2 years	2.5 years	3 years	3.5 years	4 years	4.5 years	5 years					
Date		Use WA Clozapine Initiation and titration Chart (First 4 weeks)																					
Troponin																							
C-Reactive Protein																							
ECG		As clinically indicated				As indicated																	
Echocardiogram		ClopineCentral TM recommends an Echo at Baseline, 3 mths, years 1, 2, 5 and 10. Repeat Echocardiogram as clinically indicated i.e. resting tachycardia, tachypnea, shortness of breath or hypotension																					
Clozapine / Norclozapine levels (write value)																							
Indicated date in below boxes when completed																							
Full Physical Examination booked (annually)																							
GP Letter (6 monthly)																							
Check current Care Plan (3–6 monthly)																							
DETAILS OF PERSON COMPLETING THIS FORM <i>These are suggested guidelines only, refer to the treating psychiatrist for individual monitoring requirements</i>																							
Signature or Initials																							
Name (Please print)																							